10-11 kilograms *(21-25 pounds)* /11-18 Months PURPLE

CARDIAC RESUSCITATION

Normal Vitals: HR: 80-160, RR: 20-30, Systolic BP: 72-110 mmHg, Blood Glucose > 60 mg/dl

Resuscitation Medication - (confirm concentration is as specified)	**Dose**	**Volume**
Epinephrine (1 mg/10 mL prefilled syringe) IV/IO Q 3-5 min for arrest/bradycardia[1]	0.1 mg	**1 mL**
Amiodarone (150 mg/3 mL) IV/IO for shock resistant V-Fib (or wide-complex tachycardia*)	50 mg	**1 mL**
Lidocaine (100 mg/5 mL) IV/IO for shockable V-Fib (or wide-complex tachycardia*)	10 mg	**0.5 mL**
Atropine (1 mg/10 mL) IV/IO for bradycardia unresponsive to Epinephrine[1]	0.2 mg	**2 mL**
*Adenosine (6 mg/2 mL) IV/IO **1st Dose.** For SVT (HR >180)	1.2 mg	**0.4 mL**
*Adenosine (6 mg/2 mL) IV/IO **2nd Dose.** For SVT (HR > 180)	2 mg	**0.7 mL**
Epinephrine IV/IO (1 mg/10 mL) **Push Dose - Dilute 1 mL with 9 mL Normal Saline** = 10 mcg/1 mL	10 mcg	**1 mL (Diluted)**
Electrical Therapy	**Initial[2]**	**Repeat[2]**
Defibrillation (pediatric pads preferred) Adult pads may be used anterior/posterior.	**20 J**	**50 J**
*Synchronized Cardioversion[2] for unstable tachycardia	**10 J**	**20 J**

Equipment

OPA: **60 mm** NPA: **18 F** BVM: **Child** Laryngoscope: **1 (straight)** iGel: **1.5-2.0** LMA Supreme: **1.5** King-LT: **1.0**

ET Tube: **3.5 (cuffed)** ET Depth: **13 cm** *No ETI unless unable to ventilate*

Fluid Bolus

Normal Saline **200 mL IV/IO** - **May repeat x 1 PRN**

***CONTACT MEDICAL CONTROL** **[1]CPR if HR < 60 after O2** **[2]May adjust to closest available energy setting**

10-11 kilograms *(21-25 pounds)* /11-18 Months

CONDITIONS/MEDICATIONS

PURPLE

Normal Vitals: HR: 80-160, RR: 20-30, Systolic BP: 72-110 mmHg, Blood Glucose > 60 mg/dl

Development: (12 mos) Able to cruise and beginning to walk. (15-18 mos) Uses cup well along with some spoon agility.

Condition	Medication - (confirm concentration is as specified)	Dose	Volume
Wheezing	Albuterol Nebulized (2.5 mg/3 mL)	2.5 mg	**3 mL**
	Ipratropium Bromide Nebulized (0.5 mg/2.5 mL if wheezing)	500 mcg	**2.5 mL**
Anaphylaxis	Diphenhydramine IM/IV/IO (50 mg/mL)	15 mg	**0.3 mL**
	Methylprednisolone IV/IO/IM (125 mg/2 mL)	18.8 mg	**0.3 mL**
Anaphylaxis/ Profound Distress	Epinephrine IM (1 mg/mL) ***or*** 1 Pediatric Epinephrine Autoinjector IM (**Severe symptoms only**)	0.1 mg	**0.1 mL IM**
Stridor	Racepinephrine 2.25% Nebulized (place 0.5 mL in 3 mL normal saline)	11.25 mg	**3.5 mL**
	Epinephrine (1 mg/mL) nebulized	5 mg	**5 mL**
Seizure	Midazolam IM (5 mg/mL) **Give first if no IV**	1 mg	**0.2 mL IM**
	Midazolam IV (5 mg/mL)	0.5 mg	**0.1 mL**
Fever/Pain	Acetaminophen PO (160 mg/5 mL)	160 mg	**5 mL PO**
	Ibuprofen PO (100 mg/5 mL)	100 mg	**5 mL PO**
Hypoglycemia (<60 mg/dL)	D25% (12.5 g/50 mL) **25 mL of D50% diluted with 25 mL of Normal Saline = D25% Give Slow IV**	5.0 g	**20 mL (D25%)**
	Dextrose 10% (100mg/mL)	5.0 g	**50 mL**
	Glucagon IM (1 mg/mL)	0.5 mg	**0.5 mL IM**
Pain Management	Fentanyl IV/IO/IN (100 mcg/2 mL) **(For IN, divide dose between two nostrils)**	10 mcg	**0.2 mL**
	Ketamine IN (100 mg/1 mL)	10 mg	**0.1 mL IN**
Narcotic OD	Naloxone IV/IM (2 mg/2 mL)	1 mg	**1 mL**
	Naloxone IN (2 mg/ 2mL) **Divide dose equally between both nostrils**	1 mg	**1 mL IN**
Open Fracture	Cefazolin (1000 mg/10 mL) Mix 1 gm of medication per 10 mL NS **IV SLOWLY.**	300 mg	**3 mL IV SLOWLY**
	Ceftriaxone (1000 mg/10 mL) Mix 1 gm of medication per 10 mL NS. **Administer IV SLOWLY.**	500 mg	**5 mL**

MDHHS Michigan Department of Health & Human Services | Bureau of Emergency Preparedness, EMS and Systems of Care

MICHIGAN EMSC State Partnership Program

MI-MEDIC is provided by the Michigan Department of Health and Human Services (MDHHS), Bureau of Emergency Preparedness, EMS and Systems of Care. MI-MEDIC was developed and edited in partnership with Western Michigan University Homer Stryker MD School of Medicine, Division of EMS and Disaster Medicine.

This project is supported by the Health Resources and Services Administration (HRSA) of the US Department of Health and Human Services (HHS) under the EMSC Partnership Grant. This information or content and conclusions are those of the author and should not be construed as the official position or policy of, nor should any endorsements be inferred by HRSA, HHS, or the US Government.

If you have questions or comments about MI-MEDIC, contact: EMS@Michigan.Gov

FST5 Do you need resources related to mental health challenges?
Call 1-833-34-STRONG or go to www.fst5.org

MI-MEDIC by MDHHS, WMed is licensed under a Creative Commons Attribution-NonCommercial-NoDerivatives 4.0 International License.
Based on a work at www.michigan.gov/ems.